The Psychiatrist's Double Bind: The Right to Refuse Medication

BY MAURICE DEG. FORD, J.D.

The assertion of a patient's right to refuse medication places a psychiatrist in a double bind because he or she knows that medication will often greatly relieve mental disturbance. Delaying medication until the patient is formally judged incompetent and a guardian appointed causes discomfort for the patient, the physician, staff, and other patients. On the other hand, forcing medication on a patient undermines the latter's sense of autonomy and may interfere with his or her constitutional rights, as a federal judge has ruled in the famous Boston State Hospital case. The right to refuse medication presents a uniquely intriguing case study of a need for accommodation between abstract constitutional concepts and practical realities and has opened a profound legal and ethical debate about the nature of "true freedom."

Psychiatric expert witness: A patient who comes into the hospital, who is not violent but simply says he is very depressed, apathetic, sits in a corner, refuses medications, and that patient is going to sit there for a long time, maybe years—that's a personal quiet emergency.

Federal Judge Joseph Tauro: But he's got a right to refuse that medication, hasn't he? He's got a right to sit in that corner the rest of his life. Those are two questions, you can answer either one. (colloquy during the trial of *Rogers v Boston State Hospital*, Sept. 22, 1978 [1, trial transcript 51-31])

The assertion of a patient's constitutional right to refuse medication for mental illness places a psychiatrist in a double bind. As a trained member of a caring profession, a psychiatrist knows that the administration of certain forms of medication will often greatly relieve mental disturbance. Chlorpromazine may help clear up persecuting delusions and hallucinations, and amitriptyline markedly cuts down depression, perhaps of suicidal proportions, in many patients. Lithium, too, has made an enormous difference in alleviating manic-depressive illness. Yet these medications sometimes produce unpleasant or inconvenient side effects—drowsiness, dryness of mouth, blurring of vision, transient extrapyramidal symptoms—that may lead a patient to refuse the medication. Another patient may genuinely feel that he can get along on his own without the support of medication, or, yet again, that those who are attempting to force drugs on him are part of the conspiracy which constantly bedevils him.

In recent years psychiatrists have been made aware of other considerations: the constitutional rights of their patients and the ethical obligation to respect a patient's freedom to decide. The civil rights revolution, with its previous major emphasis on rights for blacks and women and protection of the environment, is flooding in on the shores of psychiatry. Some members of the legal profession have been so zealous of their clients' rights—with allegations of "cruel and unusual punishment" and "denial of due process and equal protection"—that they have greatly aroused psychiatrists' wrath.

Many such lawyers exhibit a lack of awareness of the seriousness of their clients' precarious clinical situations. Their very idealism may allow their client-patients "to die with their civil rights on."[1] On the other hand, as Dr. Alan Stone urged at the 1978 annual meeting of the Southern Psychiatric Association, many "psychiatrists have promised more than they could deliver and pursued a crippling course of narrow-minded self-interest, actions that have contributed to the profession's dark hour now at hand" (2). Dr. Stone believes that "organized psychiatry's shortsighted response to attacks on the profession has led to a situation in which lawyers who genuinely want to improve treatment of the mentally ill now view psychiatrists as the enemy."

Any sensitive psychiatrist, I suspect, finds himself in a double bind when a patient refuses medication. Certain medication, he knows, may greatly help his patient to feel less troubled and to be more open to the benefits of psychotherapy. Yet forcing medication on a

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From the Harvard Family Law Project and the Department of Psychiatry, Harvard Medical School, and Simmons College School of Social Work.

Address reprint requests to Mr. Ford, 74 Fenwood Rd., Boston, Mass. 02115.

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[1]Personal communication from George Ainslie, M.D., Massachusetts Mental Health Center.

patient undermines his or her sense of automony. Moreover, maintaining mutual respect between doctor and patient is an essential part of forming a therapeutic alliance. When an involuntarily institutionalized patient is involved, a psychiatrist is in another kind of double bind. On the one hand, such a patient seems to have been committed by courts for treatment, the failure to give which might result in liability. On the other hand, the psychiatrist may face lawsuit if he does not respect "the right to refuse treatment" (3).

CONSTITUTIONAL ORIGINS

The constitutional origins of the right to refuse medication stem from a long-standing recognition by courts that each person has a strong interest in being free from nonconsensual invasion of his bodily integrity (4, 5). In recent years, beginning with its decision in *Grinswold v Connecticut* (6)—which declared unconstitutional a Connecticut statute forbidding the sale of contraceptives to married couples—the Supreme Court has emphasized the constitutional right to privacy. *Roe v Wade* (7), the abortion decision, also stressed a concern for human dignity and self-determination against an unwanted infringement of bodily privacy. In 1976 the Third Circuit Court of Appeals found "at least three conceivable constitutional deprivations that may accompany the involuntary administration of [medication] by state officers acting under color of law to inmates of a state institution" (8). In the first place, the involuntary administration of psychotropic drugs, which affect mental processes, may interfere with a patient's First Amendment rights to freedom of speech and association (9, 10). Second, the patient, even though committable, may not also have been adjudicated incompetent to consent to medical treatment. Third, the court ruled, "Under certain conditions [involuntary medication] may raise an Eighth Amendment issue respecting cruel and unusual punishment" (8, 9, 11).

THE STATE'S INTEREST IN MANDATING TREATMENT

As a general rule a patient has the right to decline even essential treatment through medication (12). Certainly, in the absence of a medical emergency, forced medical treatment is a tort, for which a physician may be liable for damages (13-15). As long as the public health, safety, or morals is not endangered, courts will respect a patient's decision to forego a life-saving blood transfusion (16-18). Of course, where innocent third parties may be affected, as when Jehovah's Witness parents refuse permission for their child, courts have ruled that the state's interest in preserving life outweighs the command of sincere religious belief (19). As the Massachusetts Supreme Judicial Court noted in its opinion in the now famous *Saikewicz* case (20), a "survey of recent decisions involving the difficult question of the right of an individual to refuse medical intervention or treatment indicates that a relatively concise statement of countervailing State interests may be made . . . (1) the preservation of life, (2) the protection of the interests of innocent third parties, (3) the prevention of suicide, and (4) maintaining the ethical integrity of the medical profession."

HOW ARE MENTAL PATIENTS DIFFERENT FROM OTHER PATIENTS?

What, then, differentiates the mental patient from other patients who have the right to refuse treatment? All 50 states have recognized that, under the rationale of *parens patriae*—the state acting as substitute parent—a person may be involuntarily committed, given appropriate circumstances (21). Civil commitment is, to be sure, a species of preventive detention, long anathema to our criminal law, and the Supreme Court has recognized that it involves a "massive curtailment of liberty" (22). Indeed, the Massachusetts Supreme Judicial Court has recently ruled, applying the same standard to those confined in mental institutions as to criminal defendants, that a patient's dangerousness or substantial inability to take care of himself must be proved "beyond a reasonable doubt" (23), a task clearly beyond the present predictive power of psychiatry (24). Nonetheless, some believe that "inherent in an adjudication that an individual should be committed under the state's *parens patriae* power is the decision that he can be forced to accept treatments found to be in his best interest; it would be incongruous if an individual who lacks capacity to make a treatment decision could frustrate the very justification for the state's action by refusing such treatments" (21, p. 1344).

A THRESHOLD ISSUE—THE INVOLUNTARY PATIENT

The theory that a patient committed by the courts must accept treatment rests on the assumption that he is incompetent to make a decision that such treatment is in his best interest; yet this assumption is not necessarily correct. At a commitment hearing, *only* the issue of a patient's dangerousness (to self or others) or substantial inability to care for himself is decided. Such a patient presumptively retains *all other civil rights*—the rights to vote, to freedom of speech and religion, and to decide whether to accept medication. Mental illness, moreover, is highly selective in its effects. A paranoid patient, as noted earlier, may see any attempt to force medication as part of a conspiracy. However,

a severely depressed patient, involuntarily hospitalized as a precaution against suicide, may be extremely lucid in other areas. He may read the *New York Times* and *Wall Street Journal* (even obsessively) daily and, anticipating his approaching end, draft a valid will.

Thus, in addition to a finding of dangerousness or substantial need for care, a court should ideally make a separate finding about a patient's competence to accept or refuse medication. The report of the Task Panel on Legal and Ethical Issues submitted to the President's Commission on Mental Health (25) suggested that a ruling on the medication question might ideally take place at the time of the original commitment hearing. The panel recommended that "an involuntarily committed but competent patient shall have the right to refuse medication unless the patient is an imminent danger to himself/herself or others" and that "in the latter case, there should be a hearing mechanism which can be triggered on short notice at the request of the patient or his advocate." Moreover, the panel recommended that "if an involuntarily committed patient who is also incompetent expresses a desire to refuse medication, there should be a due process hearing—on short notice—to determine the need for such medication. . . ."

A "due process" hearing does, of course, necessitate the judicialization of psychiatric decision making. The patient will have the right to be physically present and the right to assistance of counsel. If he/she cannot afford counsel, counsel will have to be appointed by the court, the panel suggested, "not less than 10 days before the hearing." This will involve delay, during which the unmedicated patient may continue to "heat up," to the discomfort of the physician, staff, other inpatients, and himself. The patient (or attorney) will have the right to confront and cross-examine all witnesses, locking himself/herself into an oppositional stance which may impede later therapy.

Making the decision about medication into such a litigious process disturbs many clinicians. They entered their profession to practice psychiatry; now they are forced to practice law—often in a distasteful, overly confrontative atmosphere. The systemic effects of making constitutional issues out of an increasing amount of psychiatric practice may be disastrous. Psychiatrists still sufficiently idealistic or courageous to work in state and often badly understaffed public hospitals—precisely those institutions where the constitutional guarantees of the Fourteenth Amendment apply—may be driven into private practice.

CRITERIA OF COMPETENCE

In determining a patient's competence to decide about medication, the central question to be asked is the extent to which his illness impairs his ability to comprehend the necessity for a given treatment. Some patients may simply be mute when asked about medication. Others may have specific hallucinations or delusions that cloud their capacity in this regard. Stone suggested the following tests for determining when a patient's refusal of medication should be respected (24, pp. 68-69).

1. There should be a burden on the reliably diagnosed severely ill person to articulate a reason for refusing treatment.
2. Those patients who are unwilling or unable to consent, or object (the so-called nonprotesting patient), should be considered as having made an incompetent refusal.
3. If the alleged patient is able or willing to state a reason for objecting to confinement, the psychiatrist should be asked to demonstrate that the refusal is irrational and is based on or related to the diagnosed illness. . . .
4. If the patient has a reason which is not a product of his illness; e.g., "I have been a Christian Scientist all my life: I do not believe in medicines or physicians." Even though a physician might consider this irrational, it is not based on his current misperception of reality, and it should therefore be considered a competent refusal.

JUDICIAL ORDERING OF MEDICATION

A court's determination that a given patient is incompetent to refuse treatment does not necessarily imply that the court should affirmatively order it. Some therapeutic modalities, such as neurosurgery or electroconvulsive treatment, are nowadays considered so intrusive that limitations on them and procedures for determining when they may be ordered involuntarily are expressly spelled out in state law[2] (26). In addition to considering the intrusiveness of treatment, the court should weigh its irreversibility, side effects (such as tardive dyskinesia), and efficacy; whether an emergency is involved; whether an inpatient or outpatient setting is necessary; and whether less restrictive alternatives to the treatment in question have been explored and rejected for valid reasons. Psychiatric testimony will be required on each of these points (25, pp. 1435-1436).

INVOLUNTARY MEDICATION OF VOLUNTARY PATIENTS

A sharp distinction is usually made between the right to medicate forcibly involuntary and voluntary patients. The Task Panel of the President's Commis-

[2]Massachusetts Department of Mental Health Regulation 181 (1973) spells out the conditions under which electroconvulsive treatment may be administered and limits the number of treatments in any one year. But a troubling question remains: What if a patient has received the statutory maximum number of treatments and comes in later in the year actively suicidal and electroconvulsive treatment is the treatment of choice?

sion on Mental Health stated in no uncertain terms that "a voluntary patient should have the absolute right to refuse treatment, and there should be a meaningful spectrum of choices of potential treatments for such patients" (25, p. 1434). However, it seems evident, as a clinical matter, that whether a patient comes into the hospital voluntarily or involuntarily does not dispose of the quite separate issue of his competence to refuse medication. Some voluntary patients are so needy and so accepting of the doctor's authority that they will consent to any medication at any dosage level and perhaps even to neurosurgery. On the other hand, the wards are full of paranoid patients, on conditional voluntary status, who sign themselves in to the hospital but adamantly refuse medication.

THE PROBLEM OF GUARDIANSHIP

A court's usual solution, after a person is found incompetent to consent to medication, is to appoint a guardian. The guardian's legal obligation is to exercise informed consent in the best interests of the patient. This makes perfect sense in theory, and it seems an essential step to those of a lawyerlike cast of mind, whose prime focus is on proper procedure. Procedure is the essence of the law; the substantive result is usually not as important to lawyers as whether it was reached by a correct mechanism. (A conviction of a person who, in fact, committed a criminal act will be overturned if he has been denied procedural due process in any significant respect.) The importance that the law—in contrast to the field of psychiatry—places on procedure is dramatically highlighted by the colloquy between Federal District Judge Joseph Tauro and psychiatric witnesses for the defense in the Boston State Hospital case (1, Oct. 29, 1979), a recent landmark constitutional ruling in the area of the right to refuse medication. The judge continually wondered why it did not occur to the doctors to secure a guardian before administering medication involuntarily (1, trial transcript 51-35-41):

The doctor: In my opinion, it's acceptable medical practice to intervene with forcible medication prior to seclusion.

Judge Tauro: Let me ask you this, going back to the depressed patient who is sitting in the corner: you think that the patient should take the medicine. The patient says, "I don't want to take it." Now, the patient is not threatening anyone, not threatening suicide, just withdrawn. When I say just withdrawn, I don't mean to minimize it as a problem, but there is no apparent danger to himself or to anyone else, at least nothing immediate. Now, at that point why isn't it a more reasonable, less restrictive alternative to impose on a staff, even an understaffed institution like Boston State Hospital, to have a petition for guardianship taken out so that determination can be made as to whether or not this individual is competent to make that decision; and if not competent then, the guardian would be appointed, and then the guardian would give the consent or withhold consent for forced medication?

The doctor: That seems to be a very reasonable alternative.

Judge Tauro: Having this reasonable alternative available, how can you take the position that the patient . . . doesn't have a right to refuse the treatment as long as he or she isn't bothering anybody else?

The doctor: What I'm thinking specifically of . . . [is] where the individual can't be reached to understand the consequences of not taking medication.

Judge Tauro: But there's no violence.

The doctor: No violence.

Judge Tauro: You and I aren't discussing a situation where you make a medical judgment that if this medication isn't taken that this person is going to smack Charlie right in the chops; we're not talking about that . . . what I'm talking about is the person just sitting in a corner withdrawn, eats meals, not starving to death, just withdrawn, doesn't want to be bothered, and you have reached the point, you say, to yourself, "She's not going to get better unless we give her some medication and bring her out of it"; she says, "I don't want to." At that point, how can you force her to take it? Don't you have to, first of all, go to a court and establish her incompetency and then deal with somebody you can talk to and who will make decisions for her?

The doctor: I find it difficult to try and think about that in general, because . . . the usual practice was if a patient refused medication, under those circumstances, to sit with the patient and attempt to somehow persuade the patient. If a patient says, "I love you, but I'm not going to take the medication," and he is eating three meals a day, I think it would occur to the hospital to say to the patient, "If you don't want to be treated, you are free to leave."

Judge Tauro: That is a voluntary patient. I think that's a reasonable reaction: "If you don't want to take my advice, go someplace else." What about the involuntary patient?

The doctor: Who was committed for treatment?

Judge Tauro: Was committed. He wasn't sentenced. This isn't punishment.

The doctor: No.

Judge Tauro: The involuntary patient was committed primarily to quarantine that patient from the outside world, hopefully going to be able to be treated and cured; you say an involuntary patient has no right to refuse treatment regardless of any situation, is *that* your premise?

The doctor: In a nonpsychiatric emergency?

Judge Tauro: Let's get back to our depressed person just sitting in the corner, does the involuntary patient have a right to refuse treatment?

The doctor: I would, looking at the practice, say that the involuntary patient in that situation would have to accept treatment.

Judge Tauro: Would have to accept forced medication?

The doctor: Given a situation of the patient who was psychotic, out of touch, as opposed to someone who was simply saying, "I understand the medication will help me get better, but I don't want any."

Judge Tauro: Take both instances, the patient who says, "Look, I like you, Doctor, I know that it will probably help me, but I just don't want the medication, I don't like the medication, I don't like what it does to me, I don't like what it makes me feel. Come up with something I like and someday I'll take it. Right now, I don't want it. I know that means I'm going to be involuntarily committed here for years." Now, that patient, do you have a right to forcibly medicate him?

The doctor: No, I would think about discharging him.

Judge Tauro: He's *involuntarily* committed.

The doctor: The involuntary commitment, as you know, can be broken by discharge. If the patient was in that good touch, was not dangerous to himself or others, and I'm just having a hard time.

Judge Tauro: What makes him in good touch if he's refusing medication and you think it necessary for him to get rid of whatever it is that caused the commitment in the first place?

The doctor: I'm thinking of the patient who is so delusional, who thinks that he is the devil and he deserves to suffer.

Judge Tauro: Involuntary?

The doctor: Involuntary, yes.

Judge Tauro: But he's not hurting anybody, not hurting himself?

The doctor: Yes.

Judge Tauro: Can he refuse treatment?

The doctor: I would be inclined to have him treated.

Judge Tauro: Forcibly?

The doctor: Forcibly.

Judge Tauro: Even though there is no emergency in terms of any danger to himself, any physical danger to himself or to another?

The doctor: In my opinion, he would do well to be treated.

Judge Tauro: We know you feel that way. Can he refuse it?

The doctor: I don't think so, sir.

Judge Tauro: Do you think that patient might consider it to be punishment to have medication forced upon him when he's not doing anything anyway, just minding his own business? He was sent there. He's just minding his own business, not hurting anybody. He gets a needle up his rear end. Do you think that patient might perceive this as punishment?

The doctor: The problem, your Honor—

Judge Tauro: Do you understand the question?

The doctor: I just want to—yes, I think I understand the question.

Judge Tauro: Answer it.

The doctor: May I make a comment?

Judge Tauro: Answer the question.

The doctor: I don't think I can, sir.

Judge Tauro: That's your answer.

In addition to the problems of communication between the psychiatric and legal professions (evident in the dialogue above), the practical realities surrounding guardianship make strict application of the theory difficult. In the first place, many patients are so disturbed as to be unable to comprehend the pendency, or nature, of the guardianship proceeding. When they receive a formal notice from the court telling them (usually in legalese) that a guardianship hearing will take place at 10 o'clock on Tuesday morning at the old Suffolk County Courthouse, it often seems meaningless. Strictly speaking, a guardian *ad litem*—a guardian to read and interpret the notice—should be appointed to assist the patient. We might call this additional person the guardian to explain and oversee the guardianship proceeding.

Second, after the final guardian is appointed, he should demand to receive sufficient medical input to make an informed judgment about the effects of various medications. A physician, knowing how fearful a paranoid patient is, may be leery of explaining the full potential of chlorpromazine, if administered over a long time, for producing tardive dyskinesia. But a guardian might validly demand that the graphic symptoms be shown to him and his ward on videotape. Guardians who discharge their duties conscientiously ask hard and time-consuming questions. Hence they are more of an annoyance to many psychiatrists than comparatively passive patients. Yet this is the very reason for their appointment.

Whom can the probate court find to serve as guardians of the incapacitated mentally ill? There is, I assure you, no long line of applicants for the job. Many patients who end up in a state mental institution have no family or friends willing to undertake the task. The judge may, of course, search around the courtroom looking for an obliging lawyer willing to serve, but there are usually no funds available to pay for such a trying obligation. And until the guardianship proceeding is completed, which may be a matter of many weeks, the unmedicated patient may continue to be out of control. For a court proceeding to be most useful, a judge should be able to act as the clinical urgency of the situation demands.

MUST "DUE PROCESS" ALWAYS MEAN JUDICIAL PROCESS?

It should be clear from the foregoing that if the appointment of a guardian (and often a guardian *ad litem*

as well) is required for the large number of incompetent patients who refuse medication, a staggering situation will result. The courts will be flooded with the guardianship petitions, and lawyers, many of them from legal service offices, will eat up precious amounts of psychiatrists' and mental health staff's time, which is already in too short supply to provide adequate patient care in most public facilities. Millions of dollars will have to be spent on such legal involvement, especially if, as the Tenth Circuit Court of Appeals has intimated, in the case of involuntarily committed patients, the "guiding hand of counsel" is required at every step of the proceedings (27). Hence the question must be raised whether any mechanism *short of judicial process* will adequately safeguard a patient's rights so as to satisfy constitutional guarantees.

In this series of articles, Gutheil, Shapiro, and St. Clair explore the attempts to grapple with this issue on the inpatient services at the Massachusetts Mental Health Center before the Boston State Hospital case was decided. At first, legal counsel for the Massachusetts Department of Mental Health argued that the voluntary or conditional voluntary patient had the right to refuse medication, except in those emergency situations where chemical restraint was required (letter from Thomas Martin to George Ainslie, M.D., May 3, 1978). Educated to think in a traditional legal framework, I at first shared that opinion. However, as Gutheil, Shapiro, and St. Clair point out, many basic clinical problems are posed by guardianship. The therapeutic alliance is no longer with the patient but past the patient, and the patient's ability to assume responsibility is inescapably undercut. At the Massachusetts Mental Health Center, the practical effects of an experiment with resorting to legal guardianship in each instance proved it unworkable in the opinion of the treating physicians and staff.

The failure to medicate during the pendency of guardianship proceedings raises some fundamental questions, because a number of conflicting rights seem to be involved: 1) the rights of the patient who refuses to be medicated (might he not sue for malpractice a psychiatrist who went along with the refusal even though he knew the patient was acting incompetently and self-destructively?), 2) the rights of his fellow patients, close by in confined quarters, who might bear the brunt of the unmedicated patient's physical and/or verbal violence, 3) the rights of the hard-pressed treatment staff.

After a searching internal review of what its policies should be,[3] the Massachusetts Mental Health Center decided in 1978 that patients may be medicated against their will in a psychiatric emergency, when there is a substantial risk of serious self-destructive or assaultive behavior. The most difficult decision-making situation is, of course, when a patient's condition stops short of being an emergency but is nonetheless critical. At the Massachusetts Mental Health Center a critical situation is defined as one in which prompt initiation of a course of psychiatric medication is necessary in order to prevent further severe suffering by the patient and/or rapid worsening of his clinical state. The Massachusetts Mental Health Center's conclusion was that in such situations, first, a Board-certified psychiatrist in the line of clinical responsibility for the patient should determine that the patient's condition is in fact critical—that in his opinion a course of psychotropic drug treatment is an essential part of the patient's treatment and that alternative treatments would be insufficient. Second, an independent outside consultant who is a Board-certified psychiatrist must come to the same determination. Third, formal guardianship proceedings should be initiated in the interim. If these conditions are met, medication may be administered involuntarily without awaiting the conclusion of the guardianship hearing. Finally, where a patient is in need of medication but is in a noncritical situation, and refuses it, the consent of a duly appointed guardian is deemed to be necessary before the medication may be given.

On October 29, 1979, Judge Joseph Tauro handed down his opinion in the Boston State Hospital case. Not surprisingly, as the tone of his dialogue with the unnamed psychiatric expert witness indicates, he came down quite hard on the psychiatric profession. Judge Tauro ruled that henceforth patients had a constitutional right to refuse medication "except where there is a substantial likelihood of . . . extreme violence, personal injury, or attempted suicide" (1, 28). His order applied to both involuntary and voluntary patients.

The right to refuse medication presents a uniquely intriguing case study of the need for accommodation between abstract constitutional concepts and practical realities, foremost among which are scarce mental health resources and the conflicting rights of other patients. The issue is a test of the ability of our judicial system to inform itself about, and to adjust to the needs of, clinical practice and the necessity for psychiatry to respect the procedural safeguards implicit in "due process of law." The procedures arrived at by Massachusetts Mental Health Center do not satisfy Judge Tauro's view of what the Constitution requires.

While recognizing the very trying conditions that exist on the wards of large, rundown, and understaffed state mental institutions, Judge Tauro remained, in the end, unpersuaded. Patients' constitutional rights to privacy, liberty, and freedom of expression, he concluded, override even the best intentions of the psychiatric profession. In the first place, patients confined to the Austin Unit at Boston State Hospital, "a drab, gloomy, poorly lighted structure in a state of chronic disrepair" (1, p. 19) with frequent "basement flooding

[3]Letter and accompanying protocol from Miles Shore, M.D., Superintendent of Massachusetts Mental Health Center, to Robert Okin, M.D., Massachusetts Commissioner of Mental Health, July 5, 1978.

and the odor of sewage through the building" (1, pp. 19-20) had few enough of the amenities of life. They were isolated from their families, if they had any family members left who cared and, as Judge Tauro was careful to remind the readers of his opinion, were "not subjects of criminal investigation, but . . . victims of fate who have been shortchanged by life" (1, p. 78). If, in addition, they refused the proffered medication, they faced the cheerless prospect of being disrobed and forcibly injected in the buttocks. Such a procedure, the court ruled, violates patients' constitutional rights to privacy and bodily integrity. To support this, he quoted from the recent decision of the Massachusetts Supreme Judicial Court in the famous *Saikewicz* case (20, p. 63):

> The constitutional right to privacy . . . is an expression of the sanctity of individual free choice and self-determination as fundamental constituents of life. *The value of life so perceived is lessened not by a decision to refuse treatment, but by the failure to allow a competent human being the right of choice.* (italics added)

One of the more intriguing aspects of the opinion in the Boston State Hospital case is its emphasis on a patient's First Amendment rights to freedom of speech and expression. It is admitted that antipsychotic medication is mind-altering. "The right to produce a thought—or refuse to do so—is as important," the judge wrote, "as the right . . . to give birth or abort" (1, p. 67). The capacity to produce ideas, even ones which many people may believe to be crazy, is one of man's most cherished freedoms. As Justice Brandeis wrote in a famous dissenting opinion over 40 years ago (29, p. 70):

> The makers of our Constitution undertook to secure conditions favorable to the pursuit of happiness. They recognized the significance of man's spiritual nature, of his feelings and of his intellect. They knew that only a part of the pain, pleasure and satisfactions of life are found in material things. They sought to protect Americans in their beliefs, their thoughts, their emotions and their sensations. They conferred, as against the Government, the right to be let alone—the most comprehensive of rights and the right most valued by civilized man.

Vincent van Gogh would probably be diagnosed today as psychotic. He certainly exhibited self-destructive tendencies by lopping off an ear. Yet would the world have experienced the glory of a sunflower so brilliantly had he been medicated? We do not know.

While establishing a constitutional right to refuse medication, Judge Tauro refused to hold the doctors at Boston State Hospital liable for monetary damages for past abuses. They did the best they could, like soldiers on the front lines, in very difficult circumstances, honestly believing in the widespread medical practice of the time and should not be charged with foreseeing the creation of a new constitutional right in 1979. Nonetheless, many members of the medical profession will protest that Judge Tauro's ruling is too restrictive, especially in requiring that a patient be formally adjudicated incompetent and a guardian appointed before medication can begin (if, of course, the guardian consents). The judge did not seem to appreciate fully how difficult it is to secure guardians for the mentally ill in Massachusetts.

Judge Tauro's opinion also raises a basic dilemma about the nature of true freedom. The psychiatric profession contends, with much evidence, that the use of antipsychotic medication and, on occasion, seclusion are, much like the law, "wise restraints which make man free." They can get many people over the hump of severe immobilization and free them up for discussion of their problems. Yet Judge Tauro is worried about the dangers of being "forced to be free" according to someone else's "better judgment," even if that judgment is the product of considerable training and experience:

> Whatever powers the Constitution has granted our government, involuntary mind control is not one of them, absent extraordinary circumstances. The fact that mind control takes place in a mental institution in the form of medically sound treatment of mental disease is not, in itself, an extraordinary circumstance warranting an unsanctioned intrusion on the integrity of a human being. (1, p. 68)

Whether or not there are appeals of Judge Tauro's decision, a profound legal and ethical debate has begun.

REFERENCES

1. Rogers v Boston State Hospital, Civil Action 75-1610 (D Mass 1975)
2. Clinical Psychiatry News, Dec 1978, p 1
3. Levick M, Wapner A: Advances in mental health: a case for the right to refuse treatment. Temple Law Quarterly 48:354-383, 1975
4. Union Pacific Railway v Botsford, 141 US 250, 251 (1891)
5. Thibault v Lalumiere, 318 Mass 72 (1945)
6. Grinswold v Connecticut, 381 US 479 (1965)
7. Roe v Wade, 410 US 113 (1973)
8. Scott v Plante, 532 F 2d 939, 946 (3rd Cir 1976)
9. Mackey v Procunier, 477 F 2d 877 (9th Cir 1973)
10. Kaimowitz v Michigan Department of Mental Health, Civil Action 73-19434-AW (Mich Cir Ct, July 10, 1973)
11. Knecht v Gillman, 488 F 2d 1136 (8th Cir 1973)
12. Cantor N: A patient's decision to decline life-saving medical treatment: bodily integrity versus the preservation of life. Rutgers Law Review 26:228, 236-238, 1973
13. Prosser WL: Law of Torts, 4th ed. St Paul, Minn, West Publishing Co, 1971
14. Winters v Miller, 446 F 2d 65, 68 (2d Cir), cert denied, 404 US 985 (1971)
15. Dunham v Wright, 423 F 2d 940 (3rd Cir 1970)
16. Estate of Brooks, 32 Ill 2d 361 (1965)
17. Osborne, 294 A 2d 372 (DC Ct App 1972)
18. Byrn R: Compulsory lifesaving treatment for the competent adult. Fordham Law Review 44:1-36, 1971
19. Application of the President and Directors of Georgetown College, Inc, 331 F 2d 1000 (DC Cir), cert denied, 377 US 978 (1964)
20. Superintendent of Belchertown State School v Saikewicz, 1977 Mass Adv Sh 2461, 370 NE 2d 417, 425 (1977)

21. Note, Developments in the law: civil commitment of the mentally ill. Harvard Law Review 87:1190, 1201-1222, 1974
22. Humphrey v Cady, 405 US 504, 509 (1972) (dictum)
23. Superintendent of Worcester State Hospital v Hagberg, 1978, Mass Adv Sh 187
24. Stone AA: Mental Health and Law: A System in Transition. NIMH Crime and Delinquency Monograph Series. DHEW Publication ADM 72-176. Washington, DC, Government Printing Office, 1975, pp 25-40
25. Report of the Task Panel on Legal and Ethical Issues: Task Panel Reports submitted to the President's Commission on Mental Health, vol 4. Washington, DC, US Government Printing Office, 1978
26. Note, Regulation of Electroconvulsive Therapy. Michigan Law Review 75:363-412, 1976
27. Heryford v Parker, 396 F 2d 393, 396 (10th Cir 1968)
28. Rennie v Klein, 462 F Supp 1131 (DNJ 1979)
29. Olmstead v United States, 277 US 438, 478 (1928), dissenting opinion